

NEPHROLOGY TEACHING SERIES

Chronic Kidney Disease

Outpatient management — the four pillars

Dialysis initiation is driven by symptoms and complications, not by eGFR alone.

Dr. Cheng • Premier Nephrology Medical Group

MS3/MS4 rotation teaching

Why it matters

CKD is common, silent, and modifiable — most of the work is outpatient.

WHAT THE CLINIC VISIT COVERS

Stage + cause — eGFR, UACR, trend

Four pillars: ACEi/ARB, SGLT2i, finerenone, GLP-1 RA

BP, volume, K, bicarb

Anemia + CKD-MBD screening

CV risk: statin, lifestyle

Vaccinate: flu, COVID, pneumo, HBV

Modality education by eGFR < 30; transplant referral as eGFR approaches 20

TEACHING PEARL

Never chart just an eGFR. CKD is CAUSE + GFR STAGE + ALBUMINURIA. eGFR 55 / UACR 800 beats eGFR 40 / UACR 10 — the risk drives the plan.

KDIGO: Cause, GFR stage, Albuminuria (CGA)

Every CKD patient gets characterized on three axes.

	A1 (UACR < 30)	A2 (UACR 30–300)	A3 (UACR > 300)
G1 (≥ 90)	Low	Moderate	High
G2 (60–89)	Low	Moderate	High
G3a (45–59)	Moderate	High	Very high
G3b (30–44)	High	Very high	Very high
G4 (15–29)	Very high	Very high	Very high
G5 (< 15)	Very high	Very high	Very high

Risk categories drive visit cadence, referral, and intensity of therapy. Very-high-risk patients need nephrology follow-up every 3–4 months minimum.

The four pillars of renoprotection

For diabetic kidney disease, aim for all four when eligible; non-diabetic CKD uses selected pillars.

ACEi / ARB

First-line for UACR > 30

Max tolerated dose

Accept Cr ↑ up to 30%

Hold if K > 5.5

RENAAL, IDNT, Captopril, AASK

SGLT2 INHIBITOR

Dapa or empa 10 mg daily

Start at eGFR ≥ 20; continue to KRT

Benefit independent of DM

Expect eGFR dip ~4 mL/min

DAPA-CKD, EMPA-KIDNEY, CREDENCE

FINERENONE

Non-steroidal MRA; T2DM + albuminuric CKD

10 mg if eGFR 25–<60; 20 mg if ≥60

K+ ≤ 5.0 at start

Additive to ACEi/ARB + SGLT2i

FIDELIO, FIGARO

GLP-1 RA

Semaglutide 1.0 mg SQ weekly

T2DM + CKD with CV/kidney risk

Weight loss bonus

FLOW (2024) — 24% RRR

Must-ask history and must-do labs

The backbone of every CKD visit.

MUST-ASK HISTORY

Cause: DM, HTN, GN, PKD, stones, obstruction
Home BP log (AM + PM readings)
Weight changes / edema / dyspnea on exertion
Urinary symptoms: frothy, hematuria, nocturia
OTC and supplements: NSAIDs, PPIs, herbal
Diet: protein load, Na⁺, K⁺ (bananas, potatoes), salt substitutes
Smoking / alcohol — modifiable risk factors
Prior kidney imaging or biopsy

LABS AT EACH VISIT (TIERED)

Every visit: BMP, UACR
Q3–6 mo (stage 3+): CBC, Mg, PO₄, bicarb
Q6–12 mo (stage 3b+): PTH, 25-OH vit D, ferritin / iron sat, albumin
Annually: lipid panel, HbA1c if DM
Once: hepatitis serologies (transplant prep), HIV, renal US if never done
Home BP — bring log; if not, teach how to log

Blood pressure in CKD

KDIGO 2021: standardized office SBP < 120 mmHg when tolerated; routine/home targets are individualized.

MEASUREMENT MATTERS

Use attended automated office BP (AOBP) when possible

Home BP: AM + PM, 2 readings each, 1 min apart

Correct cuff size; arm at heart level; quiet 5 min

White-coat HTN is real — home/ambulatory BP catches it

Masked HTN: normal in clinic, elevated at home

'120' target was derived from AOBP in SPRINT — not routine office BP

DRUG SEQUENCE

1. ACEi or ARB (never both)
2. Thiazide or loop diuretic (loop if eGFR < 30)
3. Amlodipine or other CCB
4. Spironolactone or finerenone (resistant HTN)
5. Additional: beta-blocker, hydralazine, clonidine

Avoid: dual RAAS blockade (ONTARGET — harmful)

In DKD: favor ACEi/ARB + SGLT2i + finerenone combo

Complications — screen and treat

The reasons patients feel bad and the reasons they die.

Complication	Screen	Treat
Anemia	Hgb, ferritin, TSAT (q6–12 mo stage 3+)	Iron if ferritin < 100 (TSAT < 40%) or ferritin 100–300 with TSAT < 25%. ESA after iron by shared decision (~Hb 8.5–10); keep Hb < 11.5 (KDIGO 2026).
CKD-MBD	Ca, PO ₄ , PTH, 25-OH D (q6–12 mo stage 3b+)	Treat persistent hyperphosphatemia with diet ± binders. Cholecalciferol for low 25-OH D. Reserve calcitriol/paricalcitol for severe progressive 2° HPT in G4–G5.
Metabolic acidosis	Serum bicarb (q visit stage 3b+)	KDIGO 2024: consider oral NaHCO ₃ when bicarb < 18. BiCARB trial did not confirm routine benefit.
Hyperkalemia	BMP q visit; symptoms; diet hx	Diet first. Binder (SZC, patiromer) lets you keep RAAS (AMBER). Avoid Kayexalate.
CVD risk	Lipid panel, HbA1c, smoking	Statin in most CKD (SHARP — not in dialysis). ASA if ASCVD. BP to target. Smoking cessation.

Preparing for kidney failure

Start the conversation at eGFR 30. Refer to transplant at eGFR 20.

MODALITY EDUCATION (eGFR < 30)

Discuss: in-center HD, home HD, PD, transplant, conservative care

Access planning around eGFR 15–20 (earlier if rapid decline): an AVF needs ≥ 6–8 weeks, often months, to mature (KDOQI 2019 Life-Plan)

Protect non-dominant arm veins — no PICC, no IVs from now

Patient preferences, caregiver support, home environment

Social work + dietitian involvement

Dialysis is not an eGFR threshold — it's a symptom threshold

TRANSPLANT EVALUATION

Early referral at eGFR ≤ 20 — allows preemptive listing

Pre-emptive transplant = best outcomes

Living donor > deceased donor (longer graft life)

Workup: cardiac, cancer screen age-appropriate, HLA typing, HBV/HCV/HIV

Dental clearance, vaccines UTD pre-transplant

Barriers to screen: SES, adherence concerns, active cancer (usually 2–5 y wait)

Landmark CKD trials you must know

Organized by pillar.

Trial	Year	Takeaway
RENAAL	2001	Losartan ↓ ESRD/doubling Cr 25–28% in T2DM nephropathy.
IDNT	2001	Irbesartan — renoprotection independent of BP lowering.
ONTARGET	2008	Dual RAAS (ACEi+ARB) → harm, more hyperK and AKI.
SPRINT	2015	Intensive SBP < 120 ↓ CV events 25% (non-diabetic).
CREDENCE	2019	Canagliflozin ↓ kidney failure 30% in DKD.
DAPA-CKD	2020	Dapagliflozin ↓ kidney failure 39% (incl. non-DM).
EMPA-KIDNEY	2023	Empagliflozin ↓ CKD progression 28% across broad eGFR.
FIDELIO-DKD	2020	Finerenone ↓ CKD progression 18% in T2DM on max RAAS.
FLOW	2024	Semaglutide ↓ kidney disease progression 24% in T2DM + CKD.

Clinical case

Think it through — answer is on the next slide.

VIGNETTE

A 58-year-old Black man with T2DM × 15 yr, HTN, obesity. Labs: eGFR 38 (CKD-EPI 2021), UACR 720 mg/g, HbA1c 7.8%, K⁺ 4.8, HCO₃ 23, Hb 10.9 (ferritin 90, TSAT 18%), home BP avg 148/88. On lisinopril 40, amlodipine 10, metformin 1000 BID, atorvastatin.

QUESTION: Which pillars are missing, and what's your next 3 interventions?

Clinical case — answer

Key teaching points from the case.

ANSWER

Missing: SGLT2i, finerenone, GLP-1 RA. Also anemia workup needed. (1) Add dapagliflozin 10 mg, (2) add finerenone 10 mg if K⁺ remains acceptable, (3) Iron (ferritin 90 meets KDIGO 2026 criteria), then discuss an ESA if Hb keeps falling.

TEACHING

CGA classification: G3b-A3 (very high risk). DKD pillars should be on-board when eligible. Metformin: cut to ≤ 1000 mg/day at eGFR 30–44 (he's on 2000), stop at < 30 . BP target is standardized office SBP < 120 when tolerated; routine/home readings are often managed closer to $< 130/80$. Also: retinal exam annually, vaccinate (flu, COVID, pneumococcal, HBV), modality education at eGFR < 30 , nephrology follow-up q3-4 months for very-high-risk.

REFERENCES

Chronic Kidney Disease

Guidelines

KDIGO 2024 CKD Evaluation & Management
KDIGO 2022 Diabetes in CKD
KDIGO 2021 Blood Pressure in CKD
KDIGO 2017 CKD-MBD update
KDIGO 2020 Transplant Candidate Evaluation

Landmark Trials

RENAAL — NEJM 2001
IDNT — NEJM 2001
ONTARGET — NEJM 2008
SPRINT — NEJM 2015
CREDENCE — NEJM 2019
DAPA-CKD — NEJM 2020
EMPA-KIDNEY — NEJM 2023
FIDELIO-DKD — NEJM 2020
FLOW — NEJM 2024
SHARP — Lancet 2011

UpToDate (2026 access):

Overview of the management of chronic kidney disease in adults • Proteinuria and albuminuria: evaluation and causes • Antihypertensive therapy and progression of CKD • SGLT2 inhibitors in CKD • Dialysis modality and vascular access planning